



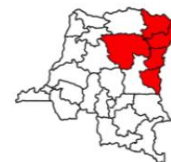
EBOLA PRESIDENTIAL FORCE TASK 17

Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°108/MVEBDB/08/30/2026

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Publication date: August 31, 2026



HIGHLIGHTS (last 24 hours)

Over the past 24 hours, 59 new confirmed cases of Ebola Virus Disease have been reported, including 19 community deaths. (EVD) cases were recorded in the provinces of Ituri (44 cases), North Kivu (8 cases), Haut Uélé (6) and Bas Uélé (1). In addition, 20 deaths within Ebola Treatment Centers were recorded. No new health zones have been affected in the last 24 hours.

ACCUMULATIVE CASE 6,100	ACCUMULATIVE DEATH CONFIRMED 2,950	LETHALITY 48.4%	PATIENTS IN ISOLATION/CTE 814	ACCUMULATIVE HEALED 1,383	TRACKING RATES CONTACTS 86.3% (Of the Day)
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Affected Provinces and Affected Health Zones



6 Provinces Affected

Haut-Uélé, Ituri, North Kivu,
South Kivu, Tshopo & Bas Uélé



60 out of 151
health zones affected
(39.7%)



Summary of key points from the last 24 hours

At the end of the day on August 30, 2026, **59 new confirmed cases**, including **19 Community deaths** from Ebola Virus Disease (EVD) have been reported in the provinces of **Ituri (44 cases)**, **North Kivu (8)**, **Haut Uélé (6)** and **Bas Uélé (1)**.

Furthermore, **17 patients** were declared recovered after receiving two negative Ebola virus (EV) test results (13 in Ituri, 3 in North Kivu, and 1 in Tshopo). However, **20 deaths within Ebola treatment centers** were unfortunately reported, including 15 in Ituri, 2 in Haut-Uélé, 2 in North Kivu, and 1 in Tshopo, bringing **the total number of deaths for the day to 39**.

As of August 30, 2026, **6,100 confirmed cases**, including **2,950 deaths**, had been reported in the DRC, representing a case fatality rate of **48.4 %**. Approximately **82.3%** of confirmed cases were concentrated in Ituri, making it the epicenter; similarly, **74.6%** of new reported cases originated from this province.

Furthermore, 60 of the 151 (39.7%) health zones in the six provinces remain affected since the start of the epidemic in the DRC. Ituri has 28 out of 36 affected health zones, followed by North Kivu (15/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34) and Bas-Uélé (3/11).

Table 1. Distribution of confirmed cases and deaths by affected province, as of August 30, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	44	5016	2,274	45.3%	28/36 (77.8%)
North Kivu	8	836	566	67.7%	15/34 (44.1%)
Haut-Uélé	6	222	97	43.7%	6/13 (46.1%)
Tshopo	0	19	9	47.4%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas Uélé	1	4	3	75.0%	3/11 (27.3%)
Total	59	6,100	2,950	48.4%	60/151 (39.7%)

Table 2. Distribution of confirmed cases and deaths by province and health zone, as of August 30, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	5016	2274	45.3%	44	12	15	27
Adja	11	1	9.1%				0
Aru	8	6	75.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	161	36	22.4%	3	1		1
Boga	2	2	100.0%				0
Bunia	1397	437	31.3%	5	0		0
Damascus	28	9	32.1%				0
Drodro	13	6	46.2%				0
Fataki	76	30	39.5%	2	0		0
Gethy	6	2	33.3%				0
Kambala	2	0	0.0%				0
Kilo	32	12	37.5%				0
Komanda	78	51	65.4%	4	3		3
Lita	218	119	54.6%				0
Logo	11	5	45.5%				0
Lolwa	22	5	22.7%	3	1		1
Mahagi	4	1	25.0%				0
Mambasa	17	7	41.2%				0
Mandima	36	21	58.3%				0
Mangala	246	127	51.6%	10	5		5
Mongbwalu	610	285	46.7%	1	0		0
Nia-Nia	215	113	52.6%	1	0		0
Nizi	662	261	39.4%	10	1		1
Nyankunde	124	35	28.2%				0
Rimba	10	4	40.0%				0

Rwampara	950	338	35.6%	5	1		1
Tchomia	57	25	43.9%				0
To ventilate	N/A	329	N/A	N/A	N/A	15	15
North Kivu	836	566	67.7%	8	5	2	7
Blessed	138	100	72.5%	2	2		2
Good	1	1	100.0%				0
Butembo	165	124	75.2%	1	0		0
Goma	1	0	0.0%				0
Kalunguta	16	7	43.8%				0
Katwa	390	257	65.9%	1	0	1	1
Kyondo	16	12	75.0%				0
Lubero	1	1	100.0%				0
Mabalako	5	4	80.0%	2	2		2
Manguredjipa	1	0	0.0%				0
Masereka	9	4	44.4%	1	0		0
Musienene	79	46	58.2%	1	1		1
Mutwanga	1	1	100.0%				0
Oicha	7	6	85.7%				0
Vuhovi	6	3	50.0%			1	1
Haut-Uélé	222	97	43.7%	6	1	2	3
Boma Mangbetu	27	14	51.9%				0
Gombari	3	1	33.3%				0
Isiro	81	33	40.7%				0
Pawa	33	19	57.6%	5	1	2	3
Rungu	1	0	0.0%				0
Wamba	77	30	39.0%	1	0	0	0
Tshopo	19	9	47.4%	0	0	1	1
Bafwasende	2	0	0.0%				0
Kabondo	3	1	33.3%				0
Lubunga	1	0	0.0%				0
Makiso- Kisangani	9	5	55.6%	0	0	1	1
Mangobo	2	2	100.0%				0
Tshopo	1	0	0.0%				0
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	4	3	75.0%	1	1	0	1
Buta	1	1	100.0%				0
Viadana	1	0	0.0%				0
Ganga	2	2	100.0%	1	1	0	1
Total	6100	2950	48.4%	59	19	20	39

*These are the confirmed deaths that occurred in the various CTEs in the province and are being broken down to be classified by health zone.

NA: Not Applicable.

** These deaths were recorded in the various CTEs of the Ituri province and are currently being assessed.

Table 3. Status of alerts notified by province, as of August 30, 2026

DPS	New Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	17	0	17	9	0	8	0	9	4
Bas Uélé	14	4	18	1	3	13	1	4	0
Ituri	656	45	701	125	43	488	2	168	105
North Kivu	755	29	784	47	28	664	1	75	23
South Kivu	ND	ND	N / A	ND	ND	ND	ND	ND	ND
Tshopo	108	0	108	2	0	106	0	2	1
Total General	1550	78	1,628	184	74	1279	4	258	133

ND: Not available.

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

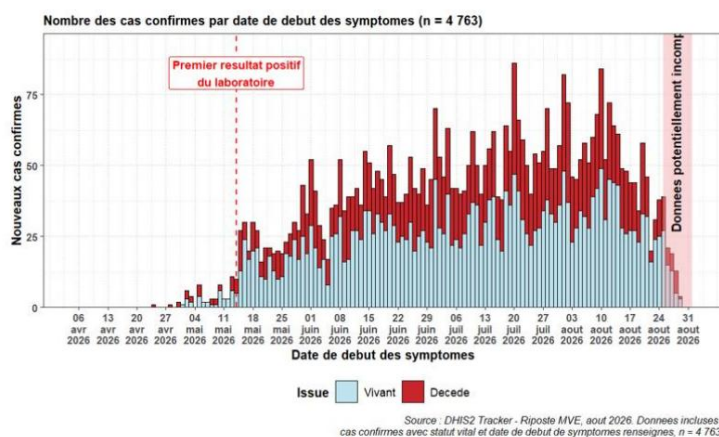
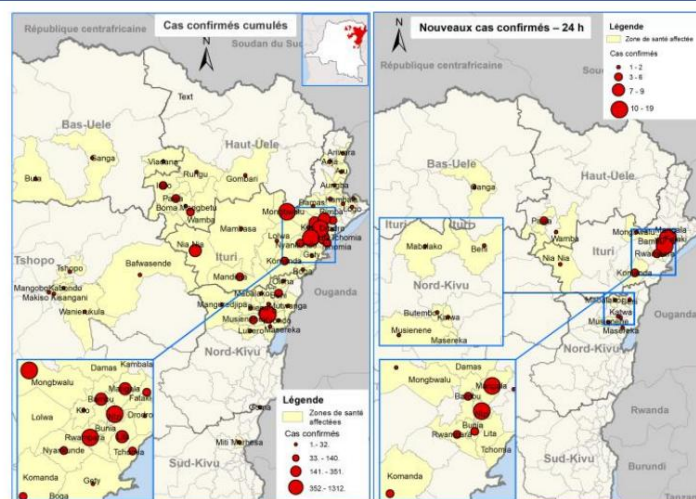


Figure 2. Spatial distribution of confirmed cases (24h and cumulative) by health zone.



1. RESPONSE ACTIONS BY PILLAR

1.1. Coordination

1.1.1. Main Actions

- Monitoring and support of response actions in all affected provinces.

1.1.2. Epidemiological surveillance

1.1.3. Main Actions

A total of 1,628 alerts were recorded as of August 30, 2026, in 5 of the 6 provinces.

Of those affected, 1,541 (94.7%) were verified. All 258 confirmed suspected cases were investigated (100%), and 133 of them were transferred to CT/CTE (Table 3).

The proportion of contacts followed up in the last 24 hours is at 86.3% (21,002 seen out of 24,330 to follow up), above the threshold of 85%: Ituri 89.4% (11,829/13,235), Tshopo 100% (455/455), North Kivu 84.8% (7,529/8,882), Haut Uélé 68.5% (1,001/1,461) and Bas Uélé 63.3% (188/297).

1.1.4. Challenges

There is persistent low contact tracing around confirmed cases in the provinces of Bas and Haut Uélé, due to insufficient teams dedicated to these activities.

1.2. Update on checkpoint and point of entry (PoC/PoE) activities

1.2.1. Main actions

- Ten (10) alerts were notified to the PoE/PoC in the last 24 hours: six in North Kivu, five of which were validated and referred to the CT (PoC Mukulia/ZS Beni, PoC Matombo/ZS Oicha, PoC Kangote/ZS Butembo and PoC Foner Kasindi/ZS Mutwanga); one in Ituri, at the PoC Irumu (ZS Nyankunde), validated and referred to the Nyankunde HGR; three in Bas-Uélé (PK 12 Isiro/ZS Viadana and PK 65 Sukisa/ZS Koteli), all invalidated after investigation.

Table 4. Status of key indicators for PoC/PoE by province, as of August 30, 2026

Indicators	Ituri	North Kivu	South Kivu	Tshopo Haut-Uélé	Bas-Uélé		Overall
Report completeness	96% (58/60)	100% (18/18)	55.5% (5/9)	100% (17/17)	36% (4/11)	55% (11/20)	83.7% (113/135)
% of functional strategic PoE/PoC	9.09% (1/11)	ND	ND	ND	ND	ND	ND
Number of people who have switched to PoE/PoC	124,031	82,924	6609	22,359	11,880	6,680	254,483
% of travelers screened	99%	97%	98% 99%		99%	98% 98.2%	
% of travelers who washed the hands	98%	97%	98% 99%		99%	98% 97.9%	
% of travelers made aware	99.5%	97%	100% 76.8%	99%		98% 96.6%	
Number of alerts notified	1	6	0	0	0	3	10
Number of live alerts validated (suspected cases)	1	5	0	0	0	0	6
Number of bodies recovered today	0	0	0	0	0	0	0
% of suspected cases transferred to CT/CTE	100% (1/1)	100% (5/5)	NA (0/0)	NA (0/0)	NA (0/0)		100% (6/6)
% of lifeless bodies swabbed/secured	NA (0/0)	NA (0/0)	NA (0/0)	NA (0/0)	NA (0/0)		
% of suspected cases investigated within 24 hours	100% (1/1)	100% (5/5)	NA (0/0)	NA (0/0)		100% (3/3)	100% (9/9)

1.2.2. Challenges

Operationalization of PoE/PoC points is suboptimal, and reporting completeness is insufficient (113 out of 135 reports: 83.7%), with particularly low levels in Haut-Uélé (36%), Bas-Uélé (55%), and Sud-Kivu (55.5%). **In Ituri**, only 1 (9.09%) of the 11 strategic PoE/PoC points is operating without interruption; the Avakubi PoC remains non-functional for security reasons, and the Pont Ituri PoC has not reported. **In Haut-Uélé**, 6 PoC points (Dusu, Apodo, PK10 Poko road, Gossamu, Salisa, and Aungba) have not reported, and the Doko PoE point does not operate on Saturdays and Sundays. **In Tshopo**, the proportion of travelers who have been made aware of the PoE/PoC points remains low (76.8%).

1.3. Laboratory

1.3.1. Main actions

- Ituri : 44 new positive results** (32 living and 12 deaths) out of 311 new samples received and analyzed (**positivity: 14.1 %**).

- **North Kivu : 8 new positive results** (3 living and 5 dead) out of 199 samples analyzed (**positivity of 4.0%**).
- **Haut-Uélé: 6 new positive results** (5 living and 1 death) out of 17 samples analyzed (**positivity: 35.2%**).
- **Bas-Uélé: 1 swab received and tested**, the result came back positive.
- **Tshopo:** 2 new samples were received, all came back negative.

1.4. Infection Prevention and Control (IPC/EDS)

1.4.1. Main actions

- **In Ituri**, 18 rings were opened out of the 41 identified (47%), 94 households decontaminated out of 120 identified (78.3%) and 4 out of 4 FOSA (100%); 62 EDS carried out out of 83 alerts and 48 bodies swabed (9 failures).
- **In North Kivu**, 11 rings were opened, 4 households were decontaminated out of 14 identified (28%), and 16 out of 20 ESSs were decontaminated (80%); 7 FOSA assessments were carried out with an average score of 53.3%. 56 bodies were swabed and 53 EDS were carried out.
- **In Tshopo**, decontamination of 3 rooms, 1 latrine and 1 shower of the Millénaire Polyclinic, of 2 rooms and 1 latrine of the Lombale CSR and of the MI pavilion of the Bafwasende HGR; briefing of 70 service providers and training of 64 hygienists; 9 bodies swabed and 9 EDS carried out.
- **In Haut-Uélé**, 1 ring opened and 4 decontaminations of living places out of 21; 4 EDS alerts received, 4 bodies swabed and 4 EDS carried out (Isiro 2, Pawa 2).
- **In Bas-Uélé**, receipt of PCI inputs in the ZS of Bili, Titule and Bondo; 3 bodies swabed.

1.4.2. Challenges

- Poor implementation of infection prevention and control (IPC) standards: **In Ituri**, ring opening remains suboptimal (47%) and household decontamination is incomplete (78.3%); there is an insufficient number of decontamination teams in Fataki, Mangala, and Nizi, and a lack of mobility in Komanda, Bambu, Mandima, and Mangala. **In North Kivu**, only 4 out of 14 identified households were decontaminated (28%). **In Tshopo**, there is a shortage of personal protective equipment (PPE) in health establishments and services (HSS), and the incinerator at Makiso-Kisangani General Referral Hospital is out of service.

1.5. Continuity of care

1.5.1. Main actions

- **In Ituri**, there were 75 new admissions to the various dedicated facilities and 81 discharges, including 13 recoveries. Thus, 524 patients are hospitalized in 978 beds, representing an occupancy rate of 53.6%.
- **In North Kivu**, 75 new admissions and 54 discharges including 3 recovered; 282 patients are hospitalized and the overall occupancy rate of beds in CTE/CT is oversaturated (128.2%; 282/220).
- **In Tshopo**, no new admissions were recorded. Seven discharges were reported, including one recovery. Seven patients are in isolation out of 25 beds, representing an overall occupancy rate of 32.0%.

1.5.2. Challenges

Low capacity and saturation: in Ituri, the CTEs of Bambu (confirmed) and Nizi (suspected) as well as the CTs of Kigonze and Lolwa (suspected) are saturated; in Haut-Uélé, absence of CTEs meeting standards in Isiro and Wamba; in Bas-Uélé, absence of CTEs meeting standards and of a partner to support care; in Tshopo, low capacity of the CTE and absence of a standardized CTE in Bafwasende.

1.6. Risk communication and community engagement

1.6.1. Main actions

- **In Ituri**, 9 people including 6 women were affected, in preparing families for regular monitoring; 31 people affected including 20 women for contact listing and household decontamination in the Nizi health zone; awareness around a confirmed case affected 42 people including 28 women in the Nia-Nia health zone.
- Communication actions around 9 recent confirmed cases (Nizi, Kilo, Nia-Nia, Afia, Tchomia and Sabe) affecting 185 people; listing of 37 contacts in Kilo, 39 in Rwampara, 112 in Lolwa and 14 in Nia-Nia with the support of CREC; 78 people sensitized to joining the EDS around 28 lifeless bodies
- **In North Kivu**, 35 CREC activities including 9 educational talks, 6 VADs and 1 mass awareness campaign were carried out; 7 rumors clarified out of 8 identified and 2 community incidents resolved.
- **In Haut-Uélé**, intensified mass awareness campaigns in churches in affected health zones; 2 educational talks in households; community dialogue in Pawa (252 people) and Isiro; 41 home visits; 1,120 households affected in Isiro; active search for 3 escaped PPLs
- **In Tshopo**, awareness-raising of 14,229 people (8,130 women and 6,099 men) in religious denominations; community dialogue in Bafwasende (48 people); briefing of 64 SUPROs; 3 articles published online; 1 rumor clarified.
- **In Bas-Uélé**, awareness-raising efforts have begun among key stakeholders and civil society in Buta and Intensification of awareness-raising in the health zones of Bondo and Ganga.

1.6.2. Challenges

Persistence of challenges at community level: in Ituri, 20 refusals of isolation, 9 bodies not swabbed due to community resistance and resistance in Bambu, Fataki and Mandima; in Haut-Uélé, resistance during the removal of remains and 10 health areas of resistance mapped out of 24.

1.7. Mental Health and Psychosocial Support (MHPSS)

1.7.1. Main actions

- **In Ituri**, 25 of the new confirmed cases of the day and 138 of the 231 confirmed cases under follow-up (59.7%) benefited from SMSPS interventions; 39 of the 74 new suspects (52.7%); 27 households accompanied, 47 PPL supported, 63 announcements of results including 25 positive and 7 recovered out of 10 followed in community (70%).
- **In Tshopo**, 4 confirmed cases under follow-up and 3 new suspects have benefited from SMSPS interventions; 3 laboratory results announced; 21 contacts supported, 4 caregivers and 24 household members supported; 2 recovered have joined the response teams.
- **In Bas-Uélé**, 1 laboratory result was announced and 3 PPLs (Patients with Leukemia Virus) received interventions. SMSPS.

1.7.2. Challenges

In Ituri, there is low coverage of SMSPS interventions around community deaths in Nizi, Lita, and Komanda; coverage is limited to 6 of the 11 health zones that reported (56%). In North Kivu, there is no reporting of data from the pillar. In Tshopo, there are no SMSPS providers in Wanie-Rukula and Bafwasende.

1.8. Logistics

1.8.1. Main actions

In Tshopo, monitoring of the construction of the isolation sites in Madula, Kabondo, and Konga-Konga, as well as the high-capacity Ebola Treatment Center (ETC); support for the mobility of the IPC, PECH, Communication, PoE/PoC, and vaccination pillars. In Bas-Uélé, deployment of teams for contact tracing, alert verification, and active case finding; reported shortages of hand sanitizer, disposable gowns, handwashing stations, and reusable aprons. In Ituri, 27 ambulances without fuel in 6 zones and 3 ambulances out of service

1.9. Security

1.9.1. Main actions

- In Haut-Uélé, police intercepted a body at PK 8 in Pawa, which had been removed from the Pawa CTE (Transient Extermination Center). In Tshopo, two police units were deployed to secure the laboratory.

1.9.2. Challenges

Securing strategic points of presence (PoPs) remains incomplete: in Haut-Uélé, the body intercepted at the Magambe PoP (Isiro Health Zone) was released by security forces without being able to be swabé (a type of body). In North Kivu, insecurity persists in burial areas, and the road leading to the Bundji cemetery (Beni Health Zone) remains impassable.

1.10. PSEA (Prevention of Sexual Exploitation and Abuse)

1.10.1. Main actions.

In Ituri, training of 48 community relays from the Bunia health zone on PSE themes (12 men and 36 women); awareness-raising of 40 people in the Lembabo/Mudzi Maria health zone and of 65 Red Cross volunteers in Aru (25 women and 40 men); continuation of the display of messages and the distribution of leaflets in the health zones of Bunia, Aru, Rwampara and Mongbwalu.

1.10.2. Challenges

- Still insufficient coverage of PSEA activities: 25% of the health zones in Ituri (9/36) know the reporting mechanism and only 2 partners (WHO and IMC) report their PSEA activities; lack of mapping of stakeholders; insufficient mass awareness materials; lack of means of transport and computer equipment for the pillar.

KEY MESSAGES

- Continued transmission of EVD, with 59 new cases including 19 community deaths and 20 intra-CTE recorded in the last 24 hours, bringing the national total to 6,100 confirmed cases and 2,950 deaths.
- Ituri remains the epicenter of the epidemic, concentrating 82.2% of cumulative confirmed cases and 74.6% of new cases reported in the last 24 hours and a greater expansion (28 of the 36 health zones affected).
- Surveillance shows good performance (100%) of suspected cases investigated and the follow-up rate (86.3%) is above the threshold of 85.0%.
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to quickly interrupt transmission chains.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



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